

ORIGINAL RESEARCH

The impact of entrustment assessments on feedback and learning: Trainee perspectives

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Abstract

Context: Assessment for and of learning in workplace settings is at the heart of competency-based medical education. In postgraduate medical education (PGME), entrustable professional activities (EPAs) and entrustment scales are increasingly used to assess competence. However, the educational impacts of these assessment approaches remain unknown. Therefore, this study aimed to explore trainee perceptions regarding the impacts of EPAs and entrustment scales on feedback and learning processes in the clinical setting.

Methods: Four focus groups were conducted with postgraduate trainees in anaesthesia, emergency medicine, general internal medicine and nephrology at McMaster University in Hamilton, Ontario, Canada. Data collection and analysis were informed by principles of constructivist grounded theory.

Results: Entrustable professional activities representing well-defined tasks are perceived as potentially effective drivers for feedback and learning. Use of EPAs and entrustment scales, however, may augment existing tensions between developmental (for learning) and decision-making (of learning) assessment functions. Three key dilemmas seem to influence the impact of EPA-based assessment approaches on residents' learning: (a) standardisation of outcomes versus flexibility in assessment to align with individual learning experiences; (b) assessment tasks focusing on performance standards versus opportunities for learning, and (c) feedback focusing on numeric entrustment scores versus narrative and dialogue. Use of entrustment as an assessment outcome may impact trainees' motivation and feelings of self-efficacy, further enhancing tensions between learning and performance.

Conclusions: Entrustable professional activities and entrustment scales may support assessment for learning in PGME. However, their successful implementation requires the careful management of dilemmas that arise in EPA-based assessment in order to support competence development.

1 | INTRODUCTION

It is widely recognised that assessment drives learning¹⁻³ and there is an emerging consensus that assessment programmes should

be designed to effectively shape learning processes.⁴ This is of central importance given the current movement towards competency-based medical education (CBME), which emphasises assessment for learning to support competence development,⁵ as well

as assessment of learning to support robust decisions regarding professional competence.⁶ In CBME contexts, programmatic assessment approaches typically integrate developmental (formative) and decision-making (summative) assessment functions: frequent low-stakes workplace-based assessments (WBAs) aim to provide meaningful feedback to the trainee to guide learning, whereas high-stakes competence decisions are intended to be made on the basis of aggregated assessment data collected over time and across multiple individual assessment moments.⁷ Research findings, however, suggest that the integration of developmental and decision-making functions in assessment design is very challenging as trainees' perceptions of constantly being judged may prevent them from focusing on learning.⁸⁻¹¹

In competency-based assessment, the concepts of trust, entrustment and entrustable professional activities (EPAs) have been introduced to bridge the gap between the theoretical construct of competence and clinical practice.¹²⁻¹⁴ Entrustable professional activities are activities or tasks that operationally define a profession and require the integration of numerous clinical competencies.¹³ As EPAs represent day-to-day clinical work, they are generally considered to be more intuitive and meaningful for clinicians than the abstract concept of competence. Therefore, competence assessment in clinical settings around the world is increasingly EPA-based,^{14,15} defining entrustment as an assessment outcome. Assessment scales have been designed to incorporate notions of trust and entrustment in order to monitor and provide feedback on trainees' competence development.¹⁶ Research findings show promising results regarding the use of entrustment scales when WBA methods are used for decision-making purposes. Crossley et al. for example, demonstrated improved reliability and increased discrimination between low- and high-performing trainees with the use of entrustment scales in comparison with conventional scales using a variety of WBA methods.¹⁷ Further, a study by Weller and colleagues showed that the integration of an entrustment scale into the traditional mini-clinical examination (mini-CEX) resulted in considerable improvement of assessment score reliability.¹⁸ However, whether the use of EPAs and entrustment scales impact developmental assessment approaches (ie assessment for learning) remains unknown. In a recent paper, Watling and Ginsburg questioned the suitability of entrustment language for formative assessment processes in view of inadvertent threats to learner self-esteem, which might impact learners' willingness to accept feedback.⁸ Similarly, a recent paper by Tsuei et al. described trainees' need for psychological safety (the need to feel free of constant self-consciousness about one's 'image of competence' in the eyes of others) as a prerequisite for learning and uptake of feedback.⁹ Given that learning is dependent on the feedback process,¹⁹ exploring whether the use of EPAs and entrustment scales impacts feedback and learning processes will provide important information about the value of these tools.

This study therefore seeks to understand postgraduate medical trainees' perspectives on how EPAs and entrustment scales affect feedback and learning processes in clinical settings.

2 | METHODS

We used a qualitative approach and focus group discussions to explore the research question.^{20,21} Data collection and analysis were informed by principles of constructivist grounded theory (CGT).^{20,22} We sought to gain a more in-depth understanding of trainees' experiences and perceptions of how feedback and learning in the clinical setting are influenced by a programme of assessment using EPAs and entrustment scales, with the overarching aim of refining the assessment theory that underpins competency-based assessment programmes. Using CGT, we specifically acknowledge our experiential (LM, MS, DBV) and theoretical (MG, LM, MS, DBV) backgrounds in workplace-based and competency-based assessment. The results are interpretations constructed through interactions between the participants in the focus groups and the shared experience of the research team, who engaged in the interpretation of the data generated.

2.1 | Setting

This study was conducted at McMaster University in Hamilton, Ontario, Canada.

The Royal College of Physicians and Surgeons of Canada (RCPSC) will transition all postgraduate programmes to CBME curricula by 2021, incorporating EPA assessments using entrustment scales. Entrustable professional activities are written across stages of training to reflect the increasing complexity of clinical tasks and responsibilities over time. At the time of this study, the general internal medicine (GIM), anaesthesia, nephrology and emergency medicine (EM) residency programmes at McMaster University were in the process of transitioning to CBME, and had implemented the use of EPAs in residency training in the preceding 7-8 months. Faculty development was identified as a cornerstone for the implementation process.

2.2 | Participants

We conducted purposive sampling²² to include all settings in which residents experienced the phenomenon being studied, including both specialty and subspecialty programmes. Focus group participants were selected according to which postgraduate year of the programme had transitioned to using EPAs, programme size, consent and availability to attend the scheduled focus group. This enabled us to collect rich data about how residents experienced the use of EPAs and entrustment scales. Recruitment emails were sent to the GIM, anaesthesia, nephrology and EM programme directors and administrators, and their residents. As we felt between-programme differences in EPA implementation would hinder in-depth exploration of experiences, we composed four programme-specific focus groups consisting of residents from, respectively, the anaesthesia (Year 1, n = 4), EM (Year 1, n = 6), GIM (Year 4, n = 3) and nephrology (Years

4 and 5, $n = 4$) programmes. The average age of the participants was 29 years; 31% were female and 69% were male. No incentives were offered for participating.

2.3 | Data collection

Focus groups were conducted during February and March 2018. Focus group discussions lasted an average of 60 minutes (range: 40–90 minutes). The development of the interview guide for focus group discussions was informed by key concepts in competency-based assessment, feedback for learning, and the use of EPAs and entrustment scales in clinical settings.^{23–25} The interview guide was updated iteratively between successive focus groups.²¹ Residents were invited to explore the impact of WBAs using EPAs and entrustment scales on feedback and learning, and to compare and contrast their experiences with other WBA tools. The interview guide included themes such as the quality and quantity of feedback and use of EPAs for learning (Appendix 1). After four focus groups, the research team had gained an adequate understanding of the concepts and their relationships and determined that theoretical sufficiency had been reached in this cohesive sample.²⁶ All focus group discussions were audiorecorded and transcribed verbatim. Identifying information was removed from the transcripts.

2.4 | Data analysis

Data analysis occurred alongside data collection in an iterative process. Applying principles of CGT, the focus group transcripts were inductively analysed using open coding and a constant comparative approach by three members of the research team (LM, DBV and MS) independently.²² Preliminary codes were cross-checked by independent coders, and discrepancies were discussed until consensus was reached. Through discussions amongst members of the research team, codes were subsequently organised into themes and relationships between these (axial coding). In the final phase of selective coding, the research team discussed the entire dataset and coding framework to conceptualise how feedback and learning processes in the clinical setting are influenced by the use of EPAs and entrustment scales in competency-based assessment programmes. Themes were consistent across the focus groups. We reached out to focus group participants to review the initial data interpretation as a way of member checking. Members from three of four focus groups provided minor comments; no significant changes in data interpretation were required.

2.5 | Reflexivity

Within our methodological approach, meaning is constructed through interaction between research participants, the data generated and the interpretation by the research team.²⁰ A research

assistant was recruited and trained to conduct the focus groups in order to minimise the potentially negative influence of having a senior physician guide the focus group discussion on information shared by the participants. The focus groups were observed by either a member of the research team or a secondary research assistant. As noted above, LM, DBV and MS performed the constant comparative data analysis. LM and DBV are internal medicine physicians who both hold Masters degrees in health professions education. MS is a cardiologist with a PhD in health professions education. Each member of the research team maintains curiosity about the educational impact of EPAs and entrustment scales. Members of the research team maintained reflexive journals during data analysis to help inform discussion about data interpretation.

2.6 | Ethical approval

Approval was obtained from the Hamilton Integrated Research Ethics Board (HiREB) at McMaster University (project no. 2018-3698). Informed consent was obtained from all participants.

3 | RESULTS

This study sought to understand how EPAs and entrustment scales impact feedback and learning in the clinical setting from the trainee perspective, and participants identified both benefits and challenges to the use of these tools. Although the use of EPAs and that of entrustment scales go hand in hand, participants seemed to clearly distinguish between the value of defining core tasks (EPAs) and that of obtaining assessment scores with entrustment scales. Specifically, residents identified different tensions raised by the use of EPAs and entrustment scales, each having a distinct impact on feedback and learning. We now present and discuss our findings in more detail. Key themes are illustrated by quotations from focus group discussions (with the corresponding focus group in brackets).

3.1 | Standardisation versus flexibility

Trainees appreciate that EPAs create a curriculum map by defining training outcomes and identifying important skills and capabilities that must be acquired during clinical training:

It does give us a goal of ... OK I need to be able to do this at this stage of my career independently, so I need to try and see as much of this as I can, to try and get as many evaluations and get comfortable with it.

(FG3)

Residents felt that EPAs need to be well defined and task-specific if they are to effectively guide their learning and selection of learning opportunities. When using EPAs for assessment purposes, however,

residents often experienced difficulties in matching their actual learning experiences to a predefined set of EPAs:

I just end up picking some random EPA that wasn't that interesting ... just because I can't find anything that matches the ones [professional activities] that we spent the bulk of our shifts on.

(FG1)

Residents reported feeling frustrated if EPAs did not capture the breadth, nuance or complexity of day-to-day clinical tasks that represented their authentic workplace-based learning. It was thus felt that focusing exclusively on EPAs carried the risk of undermining valuable learning opportunities that are not reflected in a predefined EPA. More specifically, residents were fearful of losing the experiential value of clinical rotations when using EPAs as the overarching and guiding framework for assessment:

I think if they put too much focus on us having to hit all those EPAs while off service, we'll miss a lot of the non-[specialty] specific training that we should be getting while off service, just to have a good general understanding of different specialties.

(FG3)

In line with this, residents described a concern that EPA assessments could become 'tick-box exercises', such that trainees might seek feedback for the sole purpose of ensuring that assessment requirements are met.

3.2 | Learning versus performing

Although residents felt that EPAs guided their goal setting by highlighting critical learning experiences, they also considered that EPAs helped supervisors give focused and at times constructive feedback:

I felt that with the EPA evaluations that I have done so far that at least it kind of makes people hone in or focus on a specific task and they are almost directed to give you more kind of constructive and relevant feedback.

(FG3)

Most participants described having noticed an increase in the frequency of direct observation and feedback since the introduction of EPAs; a slow culture shift was noted, whereby supervisors and trainees recognised the need to dedicate time to observing, discussing performance and providing feedback. Residents, however, also felt that the increased frequency of assessment could lead to 'assessment fatigue' and might eventually degrade the quality of the feedback conversation:

I feel these EPAs have actually caused a bit more of a detriment to the feedback system because now it's more frequent evaluation with less time for adequate feedback.

(FG2)

Participants' concerns that the EPA-based assessments might result in 'snapshot' assessments of the trainee's level of task performance were also reflected in comments regarding the importance of global evaluations, as well as the value of a longitudinal relationship with the supervisor completing the EPA-based assessment:

I did one or two of these on a 1-week rotation with the staff, but ... we talked about things at the end. So that was way more useful than the little snapshot EPA forms.

(FG3)

Furthermore, residents expressed concerns that EPA-based assessments might lead to strategic behaviours if assessment data were to be used in high-stakes decisions about progress and competence development. Trainees described feeling tempted to select assessment opportunities with the goal of 'passing' an EPA or 'looking good', rather than learning:

I can just predict, I would probably do it too, I would just cherry-pick the good experiences and be like ... remember that one patient that everything went well on, here is my EPA.

(FG4)

Once the EPAs had been checked off, participants identified a risk that similar clinical experiences might be devalued in the future in favour of moving on to other EPAs:

Now you've passed that EPA but you managed three conditions, one time each, are you really proficient at that ... and there's no real incentive then to learn about these things or read about them, you're just like: 'Oh now I've passed my acute care stuff.'

(FG3)

3.3 | Words versus numbers

Use of an entrustment scale within the EPA-based assessment framework made sense to learners because it defined the overall goal of training as a move towards independent clinical practice. However, participants did not view the entrustment scale as a significant change from prior numeric scales in terms of the information communicated by the scores regarding their competence development. In fact, some residents felt that although EPAs focused on a specific task and entrustment as an assessment outcome, the entrustment scale did not really provide them with very meaningful information about how they were progressing during clinical training:

I have a hard time seeing this as anything different than just a different numbering scale. Like in the end it's just a left to right spectrum, where on one side it's good and on the other side it's bad, and you can call it numbers, you can put words there, but in the end, it's the same Likert-type rating system in my mind.

(FG4)

Participants consistently emphasised that verbal feedback is much more important than scores received on the entrustment scale. Residents, however, also described frequent experiences in which verbal feedback did not align with the degree of entrustment that trainees received on a scale. This caused confusion in attempts to interpret the feedback and undermined its perceived value:

If someone gave me all independent but then gave me a ton of verbal feedback, well then I think they just did the administrative work ... wouldn't feel comfortable with me managing it alone.

(FG3)

Furthermore, although the verbal feedback was consistently viewed as more meaningful, some participants reported that they placed additional significance on the numeric score, thereby creating stress for themselves. Trainees reported that the use of entrustment scales impacted their sense of self-efficacy and coloured their reflections on workplace-based learning experiences: if residents did not achieve a sufficient entrustment score when evaluated using an EPA, they described feeling frustrated about their entire clinical experience that day:

I think the verbal feedback and the trust the staff put into me is way more important, but I probably stress out about numbers more. Especially when numbers have a direct bearing on whether or not I completed a task satisfactorily to my programme.

(FG1)

So sometimes there are days where if I miss an EPA it feels like oh man that was just a waste of a day even though it was a great day but like on paper it was a waste of a day.

(FG3)

4 | DISCUSSION

In this study, we aimed to understand the impact of using EPAs and entrustment scales on the feedback process and subsequent learning in the clinical setting. Our findings suggest that the use of EPAs and entrustment scales may augment existing tensions between the two assessment functions in competency-based education: developmental (assessment for learning) and decision making (assessment of learning). Our findings furthermore suggest that integrating assessment for and of learning in EPA-based assessment programmes requires the management of three key dilemmas (Figure 1). Firstly, assessment design needs to carefully balance flexibility (to allow for alignment with individual learning) and standardisation (for purposes of accountability and decision making). Secondly, both trainees and assessors need to maintain a careful balance between a focus on learning (to support development) and a focus on performance (to support competence decisions). Finally, assessment design needs to ensure a careful balance between words (having meaningful feedback conversations for learning) and numbers on an entrustment scale (documentation of performance to substantiate decisions).

Central to understanding the impact of using EPAs is the opportunity to define a specific authentic task that trainees can use to plan educational experiences. In addition, our study highlights a preconceived notion that EPAs should capture the existing workplace-based clinical experiences of trainees. There thus exists a tension between EPAs acting as a curriculum map for residents to ensure sufficient clinical experience and competence in key domains, yet also serving as a tool for documenting authentic day-to-day clinical experiences. In line with proposed guidelines for curriculum development using EPAs,¹² as well as prior commentaries,¹⁴ our findings suggest that the design of EPAs is key to their successful implementation. Entrustable professional activities need to be standardised and specific enough to guide learning experiences, yet also broad and flexible enough to reflect the wide range of residents' complex and unique clinical experiences. This requires careful balancing of holistic and analytic approaches in EPA design. This seems to be of particular importance to trainees when they try to use EPAs to guide their learning: if the EPAs are too broad, they provide no benefit in guiding goal setting or the selection of relevant learning experiences; however, if they are too narrow, other learning opportunities that are not captured may be ignored, trivialised or undervalued.

Our findings furthermore suggest that, although EPAs and entrustment scales can support trainees' goal setting and learning,

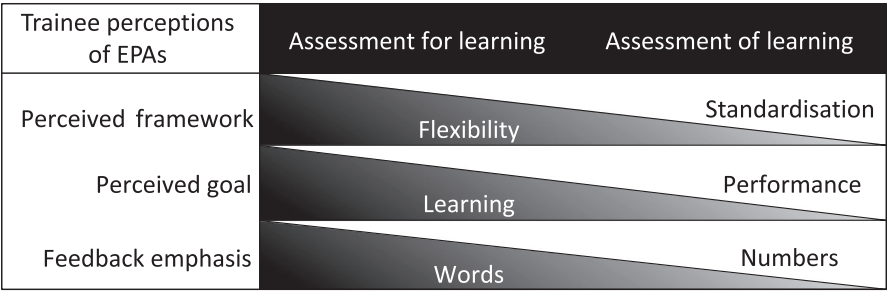


FIGURE 1 Trainee perceptions of entrustable professional activities (EPAs)

they also carry the risk of being turned into tick-box exercises. This risk is likely to be multifactorial and is explained in part by the misalignment between the EPA framework and the clinical experiences described above. In addition, some participants in this study noticed a decreased drive to continue to develop task-specific competencies once they had 'achieved competence.' The risk that EPAs might come to represent tick-box exercises is in line with well-described findings in the implementation of WBA in general.²⁷ More importantly, however, our findings could be interpreted as supportive of some critics of CBME who argue that CBME generates a focus on competent performance (demonstrating the *minimum* level of professional competence), rather than expertise or commitment to lifelong learning.²⁸ Given that assessment drives learning, it could be hypothesised that framing training outcomes around entrustment rather than expertise might affect competence development and learning outcomes in unintended ways. This may be particularly true if residents and supervisors embrace a culture of performance rather than improvement.^{8,29} Previous research findings have shown that the learning culture in medicine is complex, and that significant value is placed on autonomy and efficiency in task performance,³⁰ which may conflict with a learning culture centred on direct observation and feedback. In fact, our findings suggest that the use of entrustment language, with its focus on autonomy and unsupervised practice, may in fact reinforce a 'performance culture' at the expense of a focus on continuous development and learning. We feel that our findings provide empiric evidence for the issue raised by Watling and Ginsburg⁸ about entrustment scales inadvertently undermining the efficacy of feedback by threatening learners' self-esteem and identity. Trainees clearly expressed a desire to be successful in achieving the EPA and felt significant anxiety and frustration when use of the entrustment scale did not result in a maximum score reflecting the achievement of 'competence'. Participants did, in fact, express concerns about the impact of the entrustment scale, irrespective of its anchors, on their self-efficacy, motivation and stress levels. Assessment systems based on EPAs and the inherent focus on the assessment outcome (ie entrustment) may thus enhance learner perceptions of continuously being judged and of assessments being summative rather than formative, thus undermining the learning potential of an assessment programme based on trust and psychological safety. Based on findings from our study, de-emphasising the role of the scale and focusing on narratives and meaningful feedback conversations in the day-to-day assessment of task performance might create the psychological safety that residents need to be able to seek and use feedback for learning.

4.1 | Implications

Findings from our study draw attention to elements in assessment design that may impact the efficacy of EPA-based assessment for learning. For example, although a transparent path for learner promotion through the stages of training and specific competencies

has been endorsed,⁵ it is possible that setting granular expectations for each individual EPA (eg complete this EPA x number of times, in x number of situations, at x level of performance) may enhance perceptions of EPAs as tick-box exercises, and even more so if EPAs are not well aligned with actual learning experiences. Given that EPAs cannot perfectly capture all day-to-day clinical experiences, building flexibility into the EPA programme (or reducing granularity) for monitoring and assessing competence development may enhance alignment between learning and assessment. Furthermore, in order for feedback to truly support competence development, single 'snapshot' feedback moments must be aggregated and reflected upon in meaningful ways.³¹ Stimulating learner reflection over multiple EPA-based assessments can provide a longitudinal and more meaningful view of development³² and strengthen learners' confidence, motivation and well-being.^{33,34} Thus, in order to enhance residents' sense of competence in CBME, in line with programmatic assessment theory,⁷ EPAs should be clearly integrated such that decisions on levels of entrustment are made on the basis of rich, aggregated feedback data and multiple assessments rather than at every individual assessment moment.

4.2 | Limitations and future directions

There are a few limitations to the present study. Assessing trainee perceptions of learning and feedback during the early implementation of EPAs and entrustment scales offers a unique perspective; however, residents' experiences with these tools were limited to the preceding 7 months. Despite this, our participants had accrued sufficient experience by the time of the study to generate rich discussion during the focus group sessions. This study provides information regarding the early phases of implementation. Some reactions may be related to change avoidance and lack of clarity around assessment integration and the decision-making process. Overall, perceptions may change over time as supervisors and trainees accrue more experience with these tools. Furthermore, institutional culture is likely to continue to shift as EPAs and entrustment scales are incorporated into all postgraduate medical education (PGME) assessment programmes. Our study was a single-centre study. This may limit its generalisability to other contexts. Future research involving multiple sites and programmes across Canada, and internationally, would be valuable to understand differences in the implementation of EPAs and entrustment scales, and to explore if and how these differences impact trainee learning. Finally, our study focused on the trainee perspective. Institutional culture and local buy-in are likely to have significant impact on the successful implementation and subsequent learning value of EPAs and entrustment scales. Future studies to examine institutional, faculty staff, and educator perspectives on the impacts of using EPAs and entrustment scales in WBA may further enhance our understanding of their utility in assessment for learning. These studies will be important as more programmes,

institutions and national accreditation bodies encourage the transition to CBME using these tools.

5 | CONCLUSIONS

Ongoing commitment to change at both the individual and systems levels is required in order to create the assessment culture we are aiming for in CBME. The findings of our study illustrate that EPAs and entrustment scales may support assessment for learning in PGME. The use of EPAs and entrustment as assessment outcomes, however, can also intensify existing tensions between assessment of and for learning in competency-based assessment programmes. Future research on how to best manage the key dilemmas in EPA-based assessment programmes is required to explore if and how we can achieve the desired state: a longitudinal programme of assessment to guide high-stakes decisions, embedded in a culture of learning for high-quality performance.

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CONFLICTS OF INTEREST

None.

AUTHORS' CONTRIBUTIONS

LM was responsible for conceiving and conducting this study, data analysis and the development of the manuscript. MS contributed to the conception of this study, data analysis and manuscript development. DBV participated in data analysis and manuscript development. DR participated in data collection, iterative updating of the interview guide, and manuscript development. MG supervised the conception of the study, the data analysis, and manuscript development.

ETHICAL APPROVAL

This study was approved by the Hamilton Integrated Research Ethics Board (HiREB) at McMaster University (project no. 2018-3698).

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APPENDIX 1

ENTRUSTMENT AS AN ASSESSMENT OUTCOME: LEARNER PERSPECTIVES

Research question

From the perspective of postgraduate medical trainees, what pre-conditions need to be met to ensure that assessment leads to learning when using entrustable professional activities (EPAs)?

- Sub-question: Is there an impact on postgraduate medical trainees' self-efficacy when using trust and entrustment as an assessment outcome?

Explanation of the research

We are conducting this research study in order to explore your experiences with EPAs, entrustment scales, feedback during these assessment moments, and progression through your training. We will focus on the quality of feedback you have received during these assessments and whether and how these assessments impact your learning process. We hope this research can be used to inform the design of EPA-based competency-based medical education curricula to achieve 'assessment for learning'. Any information shared during this interview is considered strictly confidential.

This conversation will be audiotaped and transcribed. In the transcription, all individual identification (name) will be deleted (and replaced by a number).

Question 1

ASK: What is your understanding of workplace-based assessment (WBA) and what do you perceive as the role of WBA in your training?

Question 2

ASK: For you, what constitutes meaningful feedback or what do you expect from feedback in WBA?

ASK: What determines for you whether you accept or reject feedback? Can you think of experiences in which you received feedback that you disregarded?

Question 3

ASK: Can you share some experiences of WBA prior to the implementation of EPAs and entrustment scales?

Question 4

ASK: Let's have a look at EPAs and entrustability scales and their use in WBA. Can you share some experiences of being assessed using EPAs using an entrustability scale?

Question 5

ASK: How were either of these tools (ie EPAs and entrustment scales) used to guide feedback?

Prompts:

- Kinds of activities assessed?
- Written or verbal feedback? What was the focus of the feedback?
- ASK: How does this tool enhance/hinder provision of feedback? What works well, and what doesn't? How does this tool help to make feedback useful?
- ASK: What did you do with the feedback (changed practice, prompted reading, no influence at all)? Why?

Question 6

ASK IF NOT COVERED: Are there specific differences you've noticed in your experiences of WBA depending on the tool used?

Question 7

ASK: As the receiver of the feedback, does it mean something different to you when you receive a numerical rating in comparison with how much a faculty member trusts you to perform a task?

Question 8

ASK: Does either tool help you to develop an action plan or set goals for your learning?

Question 9

ASK: Do you ever feel an alliance or partnership with the staff you are working with, not only in the care of patients, but regarding your

development? What factors contribute to that partnership? *Do you feel that the tool affects the way you work together with staff, and if so, how?*

Summary question

[To summarise what was discussed; difficult to write in advance]

To summarise, _____. Do you feel this summary reflects what we have discussed today?

Wrap-up question

Those are all the questions I have written down. Is there something that I didn't ask you about that you think is important for me to know about your experience with these two tools in workplace-based assessment?

Thank you for your time today.

END